ROYAL FITNESS CLUB

RFC PREMIUM GUIDE #06

COMPLETE PEPTIDE PROTOCOL BIBLE

GHRP, GHRH, IGF-1, BPC-157 & TB-500 Complete Reference

6 Chapters · Advanced Guide · Science-Backed · Harm Reduction

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DISCLAIMER

This guide is for educational and harm-reduction purposes only. It does not constitute medical advice. Consult a qualified healthcare professional before starting any supplementation or pharmaceutical protocol. Individual results vary.

CHAPTER 1 — WHAT ARE PEPTIDES?

"Peptides are the language your cells use to communicate. Learning this language lets you speak directly to your body's growth systems."

Peptides are short chains of amino acids (2–50 residues) that act as signalling molecules in the body. Unlike anabolic steroids, which directly bind to androgen receptors, therapeutic peptides typically work by stimulating the body's own hormone production pathways. This makes them both more targeted and, in most cases, safer in terms of hormonal suppression profiles. Growth-hormone-releasing peptides (GHRPs) and their analogues represent the most researched category for performance and body composition enhancement.

Peptide Class	Mechanism	Primary Effect	Administration
GHRP (e.g. GHRP-6, Ipamorelin)	Ghrelin receptor agonist	GH pulse stimulation	SubQ injection
GHRH (e.g. CJC-1295)	GHRH receptor agonist	Amplifies GH pulse	SubQ injection
IGF-1 LR3	IGF-1 receptor direct	Protein synthesis, muscle growth	SubQ/IM injection
BPC-157	Multiple (tendon, GI, CNS)	Healing, anti-inflammatory	SubQ, oral, or IM
TB-500 (Thymosin Beta-4)	Actin filament binding	Tissue repair, flexibility	SubQ injection
Melanotan II	MC1-R / MC4-R agonist	Tanning, libido, appetite	SubQ injection

CHAPTER 2 — GHRP FAMILY PROTOCOLS

GH-Releasing Peptide Comparison

GHRP	Dose	Hunger Effect	Cortisol Effect	Best Use Case
GHRP-2	100–300mcg	Moderate	Moderate increase	Maximum GH pulse strength
GHRP-6	100–300mcg	Strong (very hungry)	Moderate increase	Bulking — hunger useful
Ipamorelin	100–300mcg	Minimal	Minimal increase	Cutting or lean bulk
Hexarelin	100–200mcg	Moderate	Low	Most potent GHRP available
MK-677 (oral GHRP)	12.5–25mg	Moderate	Minimal	Oral convenience, 24h GH elevation

■ ***Ipamorelin is the most selective GHRP — it causes GH release without elevating cortisol, prolactin, or ACTH. It is the safest choice for long-term use and cutting protocols.***

Standard Daily GHRP Protocol

- Injection 1: 6:30 AM fasted — GHRP + GHRH. Do not eat for 20 minutes before or after
- Injection 2: 30 min post-training — GHRP + GHRH. Amplifies anabolic window
- Injection 3: 11 PM pre-sleep — GHRP + GHRH. Maximises overnight GH pulse
- Each injection: 100mcg GHRP + 100mcg GHRH in same syringe (compatible)

CHAPTER 3 — GHRH ANALOGUES

GHRH Analogue	Half-Life	Dose	DAC Version	Key Difference
CJC-1295 (no DAC)	30 min	100mcg per injection	No	Pulsatile release — mimics natural pattern
CJC-1295 (with DAC)	7–8 days	2mg per week	Yes	Continuous elevation — blunts natural pulses
Sermorelin	10–20 min	200–500mcg nightly	No	Most natural, weakest signal

■ ***CJC-1295 without DAC is strongly preferred for performance use. The DAC version creates a "GH bleed" that disrupts the natural pulsatile pattern associated with optimal body composition.***

- Always pair GHRH with a GHRP — they work synergistically (4–10× amplification)
- Do not use GHRH or GHRP within 2 hours of eating — insulin suppresses GH release
- Store reconstituted peptides in the refrigerator; bacteriostatic water extends shelf life to 4 weeks

CHAPTER 4 — IGF-1 & GROWTH FACTORS

"IGF-1 is the downstream effector of GH — the molecule that actually enters muscle cells and drives protein synthesis."

Insulin-Like Growth Factor 1 (IGF-1) is produced primarily in the liver in response to GH stimulation. It mediates most of the anabolic effects of GH — muscle protein synthesis, satellite cell proliferation, and fat oxidation. Exogenous IGF-1 can be used to amplify these effects beyond what GH stimulation alone achieves.

Form	Half-Life	Dose	Timing	Key Action
IGF-1 LR3	20–30 hrs	20–50mcg/day	Post-training, subQ	Systemic muscle protein synthesis
IGF-1 DES	20–30 min	50–150mcg local	Intra-workout, local IM	Site-specific muscle growth
PEG MGF	24–36 hrs	200mcg 2× per week	Post-training, subQ	Satellite cell activation

■ ***IGF-1 LR3 cycle length should be limited to 4–6 weeks maximum. It does not suppress HPTA but may desensitise IGF-1 receptors with prolonged use.***

CHAPTER 5 — HEALING PEPTIDES: BPC-157 & TB-500

BPC-157 Protocol

Indication	Dose	Route	Duration	Expected Result
Tendon/ligament injury	250–500mcg	SubQ near injury	4–8 weeks	Accelerated tendon repair
Gut/GI issues	250mcg twice daily	Oral solution	4–6 weeks	Mucosal healing, IBS improvement
Joint pain	250–500mcg	SubQ near joint	4–6 weeks	Anti-inflammatory, cartilage support
Muscle tear	500mcg	SubQ near site	6–8 weeks	Enhanced satellite cell activity

TB-500 Protocol

Phase	Dose	Frequency	Duration	Notes
Loading	2–4mg	2× per week	4–6 weeks	SubQ injection anywhere on body
Maintenance	2mg	Once per week	Ongoing as needed	Full body systemic effect

■ ***BPC-157 + TB-500 combined is considered the most powerful healing peptide stack. Many bodybuilders use this combination during injury rehabilitation to return to training faster.***

CHAPTER 6 — STACKING GUIDE & SAFETY

Goal	Primary Peptides	Supporting Peptides	Duration
Lean muscle mass	Ipamorelin + CJC-1295	IGF-1 LR3 (4 wks)	12–16 weeks
Fat loss	GHRP-2 + CJC-1295	T3 (optional)	12 weeks
Injury recovery	BPC-157 + TB-500	None required	6–8 weeks
General anti-ageing	Ipamorelin + CJC-1295	MK-677 nightly	Year-round (cycled)
Sleep optimisation	Ipamorelin pre-sleep	MK-677 nightly	8–12 weeks

- Do not run peptide cycles longer than 12–16 weeks without a 4–8 week break
- IGF-1 use requires blood glucose monitoring — it can cause hypoglycaemia
- Monitor for water retention (GH effect) — usually resolves after Week 2
- Rotate injection sites to prevent lipohypertrophy (small fat deposits)
- Peptide-related side effects are generally mild and dose-dependent — reduce dose first